

Pharmacy Law and Ethics (Phar 4243)



Part I: Pharmacy Ethics

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Ethics : Topics to be covered

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- Basic concepts on ethics (Definition, Classifications)
- Ethical theories (Teleological and deontological theories)
- Ethical principles and moral rules
- Ethical issues in health care:
 - Rationing;
 - Assisted suicide
 - Human drug experimentation
- Ethical dilemma and framework for ethical analysis
- Professional ethics

Topics ...

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- Ethical codes:
 - FIP standards of ethical practices;
 - Code of ethics for pharmacists practicing in Ethiopia
- Health professional–Patient relationship:
 - Consumerism versus paternalism; patients' rights;
 - Moral rights versus legal rights to health care
 - Health care practitioners' duty to their patients

Learning Objectives

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- At the end of the lesson students will be able to:
 - ✓ Define terms: Ethics, Profession, Ethics, Code of ethics.....
 - ✓ List characteristics of profession
 - ✓ Describe difference and similarity b/n ethics and law
 - ✓ Describe ethical theories and principles
 - ✓ Compare Ethiopian code of ethics with FIP
 - ✓ Identify determinants for unethical practice
 - ✓ Describe ethical issues in health care and develop framework to analyse ethical dilemmas

1. Introduction to Ethics



Introduction

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- **Pharmaceutical jurisprudence:**
 - Science or philosophy of law governing pharmaceutical sectors or services
 - **Legal system** of law or the **body of law** applied in the pharmaceutical areas
- **Pharmaceutical ethics:**
 - An ethical behavior which governs pharmaceutical practice
 - Values and **ethical principles** governing pharmacy practice encouraging right human conduct

Introduction: What is Ethics?

- **Ethics;**
 - Derived from Greek word “*ethos*”: “**Custom**” or “**Character, practice, attitude and motive**”.
 - Branch of philosophy dealing with **standards of conduct** and **moral judgment**.
 - Method of inquiry that assists people to understand the morality of human behavior.
 - i.e. it is the study of morality

Introduction: What is ethics...?

- Ethics concerned with systematic study of value concepts
 - What ought to be, what is *right*, or *wrong*, *good* or *bad*.
 - It also refers to the *expected standards* as described in the group's code of professional conduct
 - It is **norms of conduct** that distinguish between **acceptable** and **unacceptable behavior**

Introduction: What is ethics...?

- **Health ethics:**

- It teaches us how to judge accurately the moral **goodness or badness** of human action in health
- Interdisciplinary field of study and practice that seeks specifically to understand the values undergirding decisions and actions in:
 - Health care
 - Health research and
 - Health policy
 - To provide guidance for action when these values conflict

Introduction: What is ethics...?

- **Medical Ethics:**
 - ✓ Branch of ethics that deals with **moral issues in medical practice**
 - ✓ Ethical issues that arise in the clinical context related to the care of specific patients
 - ✓ Field of applied ethics, the study of moral values and judgments as they apply to medicine

Ethics...

- **Health care ethics:**

- AKA “medical ethics”
- Application of the core principles of bioethics to medical and health care decisions.
- It is a multidisciplinary lens through which to view complex issues and make recommendations regarding a course of action.

Ethics...

- **Bioethics:**

- Branch of applied ethics that studies the philosophical, social, and legal issues arising in medicine and the life sciences
- Ethical issues arising from the creation and maintenance of the health of all living things
- It is chiefly concerned with human life and well-being, though it sometimes also treats ethical questions relating to the nonhuman biological environment

Ethics Vs Morality

- **Morality:**
 - Concerned with the distinction between **right** and **wrong**.
 - What people believe to be right and good
 - Related to **individual's own principles** regarding right & wrong
 - Often used in connection with the ways in which individuals conduct
 - Their personal, private lives
 - Often in relation to personal financial probity, lawful conduct and acceptable standards of interpersonal behavior (including truthfulness, honesty, and sexual propriety).

Ethics Vs Morality...

- **Ethics:**

- Science of moral principles that govern an individual's actions
- Related to **rules provided by external sources**
 - **Example:** Code of conduct in work places, principles in religion

“Do Good and Avoid Evil”

Ethics Vs Morality...

- Moral act is a human act which involves some principles of moral law;
 - If conformity with moral law, it is called **good**
 - If opposed to moral law, it is called **bad**
- Moral act is one that is carried out with at least some degree of knowledge and freedom, proceeding from man's rational nature.
- **Ethics** is careful and systematic reflection on and analysis of moral decisions and behavior

Categories of Ethics

- **Descriptive ethics :**
 - Related to *description of the values and beliefs* of various cultural, religious or social groups about health and illness
- **Normative ethics:**
 - Study of human activities in a broad sense in an attempt to identify *human actions* that are right or wrong and good and bad qualities.
 - Study of *what makes actions right or wrong*
 - What makes situations or events good or bad and what makes people virtuous or vicious

Categories of Ethics...

- **Analytical ethics :**
 - Analyzes the meaning of moral terms.
 - Seeks the reasons why action or attitudes are either wrong or right.

Categories of ethics: Group Discussion

- Identify the **descriptive** and **normative** section of the following cases and conduct moral analysis
 1. Patients in pastoralist region **believe** that capsules are more effective than tablets (highly adherent for capsules)
 2. Ethnic group in SNNPR **extracts** the milk teeth of their teenagers when reach to adult age.
 3. Patients in northern part of Ethiopia **believe** that Injections are more effective than any oral preparations; especially tablets and capsules

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Profession and Professionalism

- **What Does Profession and professional Means to You?**
- **What are the criteria's (characteristics) of profession ?**

Take a few minutes and write down your thoughts

As a definition or description?

What is Profession?

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- Form of **employment** or **field** that is **respected** in the society and that requires **extensive study** and mastery of **specialization in knowledge**
- It is an **occupation** that requires special knowledge and skilled preparation.
- An employment not mechanical & requiring some degree of **learning**.
- **Professional**: a person doing or practicing something as a full time occupation or for a payment or to make a living

Importance of Ethics

- To help health professional identify moral and ethical issues,
- To know what is right and wrong about what should and should not be done for and to client,
- To know and respect the issues of human rights, personal and civic

Characteristics of Profession

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- ◆ Specializing in **knowledge**
 - Rigorous training and schooling, problem-solving skills
- ◆ Providing essential **services** to the society
 - Deriving force: not fortune but rather the ministering
 - Provide service in the interest of the patient
- ◆ High degree of self-control of behavior/standard of conducts
 - Codes, guidelines, oaths, etc

Characteristics of Profession...

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- ◆ Mutual trust b/n professionals and the patron

 - Information asymmetry

- ◆ Social sanction

 - Professionals given exclusive right to practice the profession with full responsibility

- ◆ Representative body of practitioners/Association (optional)

Characteristics of Profession...

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Is pharmacy profession or trade?

➤ Criteria?

“Constituting those **attitudes** and **behaviors** that serve to *maintain patient interest above pharmacists Self-interest*”

Trade Vs. Profession

- What is the difference b/n **making a living from selling one's professional services** and **from buying and selling of goods** ?
- Is Pharmacy a **trading** or **profession**?
 - Particularly **retail** service

Difference b/n trade and profession

- **Trade person;**

- ☞ Concerned principally with **profitability**

- ☞ Holds the **customer to be the best judges of what they want & seeks to satisfy their demands**

➔ **“Let the buyer beware”**

- **Professional People;**

- ☞ Judges what is best for their clients/customers

- ☞ Provide services to practitioner's own financial disadvantage

◆ Why is so much expected from the professional?

➡ **Profession is the foundation of a society!**

Law Vs Ethics

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- **Law;**
 - Societal **rules or regulations** that are **obligatory** to observe
 - Set of principles **enforced** by the government
 - Governed practice of healthcare professionals

Comparing Law and Ethics

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- **Similarity**

- Both share the social purpose of **encouraging the right conduct**
- Both ethics and law are normative frameworks
 - i.e. they define how people ought to act

- **Difference**

- **Enforcement;**

- ✓ **Law:** Sovereign power of the state

- Are **mandatory** to all citizens (have risk of liability)
 - risk civil or criminal liability

- ✓ **Ethics:** without the intervention of government

Laws Vs Ethics: Difference

Law

- Rules of human conduct binding all persons in a state or nation
- May prevent one from harming others, but it can't force someone to help the needy
- If law is broken the violator is subjected to legal prosecution which may include fine or imprisonment or both depending on the severity of crime committed
- It is something that you must obey
- Prevents pharmacist from selling substandard drugs, but cannot prevent him from selling drugs at a lesser price than his fellow pharmacist
- Deals with actions that are punishable

Ethics

- Rules by which a profession regulates actions and sets standards for all its members
- Helping the needy is a value attributed by ethics
- If ethics are breached the violator is subjected to loss of professional privilege e.g. his license may be suspended, he may not be able to enjoy the honor of him being a professional
- Ethics expects a sensible & responsible behavior towards society.
- Selling drugs at reasonable rate and not getting into cut throat competition by selling drugs at cheaper rates is a part of ethics
- Deals with right or wrong

Impacts of Ethical Practices by Pharmacists

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□ **Impact on other health professionals**

- Foster team work and ensure better outcome of therapy
- Pharmacist gain better acceptance/recognition

□ **Impact on fellow professionals and the profession**

□ **Professional:**

- Develop better confidence,

- Become more committed

- **Profession:** guarantees continuity

Impacts Cont'd

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- **Impact on the patient and public**
 - Contributes to better treatment outcomes
 - Minimize risk and cost
 - Enables maintaining a healthy society

Ethical Theories

- Tools for ethical guidance that can be adopted in analyzing the range of ethics that can affect:
 - Societal culture
 - Medical and healthcare management
 - Individuals' growth
- Provide a coherent system of thought about what constitutes ethical action and tend to be abstracts
- **Helpful in addressing ethical issues** in two key ways:
 - ✓ Explain why the issue at hand is an ethical issue, and
 - ✓ Justify why one course of action ought to be preferred over another

Ethical Theories: Ideas and Actions

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- **Deontology ethical theory :**
 - Duty driven theory
 - Rule based theory
 - Non-consequentialism theory
- **Teleological ethical theory:**
 - Consequentialism
 - End based actions theory

Ethical Theories....

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1. Deontology /Duty driven/Non-consequentialism/Theory

- “*Deontology*”
 - Drived from the Greek origin name “*deon*”
 - Meaning: “**Duty**”, *Logos*, “**Science**.”
 - Introduced by **Immanuel Kant**, (1724-1804) the 18th century German philosopher from Prussia
 - Proposes that the rightness or wrongness of an action depends on the *nature of the act* rather than its *consequences*
 - An action is considered morally good because of some characteristic of the **action itself**, **not because the product of the action** is good.

Ethical theory...

- Considers the way that things are done rather than focusing just on the consequences
- This theory holds that you are acting rightly when you act according to duties and rights
- Rightness and wrongness of act
 - Inherent in the **act** it self
 - Independent of the **consequence**
 - **Process** matters more than the **result**

Ethical Theories....

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- Looks to one's obligation to determine what is ethical and answers the question:
 - ✓ What should I do and why should I do it?

Example:

- Breaks of confidentiality (HIV information to partner)
- Respect for person

“The means justify the end”

Ethical Theories....

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2. Teleological/Consequentialism/End based theory/

- “*Teleology*”:
 - Originated from a Greek name ‘*telos*’
 - Meaning: “**End or Consequence**”, “*Logos*”, “**Science**”
- Primarily concerned with the ethical consequences of particular actions
- It looks to the *consequences of an action* in judging whether that action is right or wrong

Ethical Theories....

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- Actions are determined and justified by the consequence of the act:
 - *“The result matters most”*
 - *“The end justifies the means”*
- Teleological theories stress the importance of motives behind a conduct next to an outcome.
 - **Example 1:** Informed consent
 - ✓ Improved compliance and outcome
 - **Good Consequence**
 - **Example 2:** Few cases of vaccine-induced paralytic polio after oral polio vaccination

Ethical Theories....

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□ Utilitarian (Utilitarianism) Ethics:

- Well known consequentialist theory
- Governing ethical theory in public health
- Holds that the proper course of action is the one that maximizes utility, specifically defined as maximizing happiness and reducing suffering
 - Considers the greatest good for the largest number of people
- Utilitarian hold that no action in itself is good or bad, the only factors that make actions good or bad are the *outcomes, or end results* that are derived from them.

Ethical Theories....

- Moral worth of an action is determined only by its resulting outcome
 - Although there is debate over how much consideration should be given to actual consequences, foreseen consequences and intended consequences.
 - Two influential contributors to this theory are [Jeremy Bentham](#) and [John Stuart Mill](#).
 - In A Fragment on Government Bentham says; ***‘it is the greatest happiness of the greatest number’***
 - Decisions are chose based on the greatest amount of benefit obtained for the greatest number of individuals

The Identity Argument

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- **Premise 1:** What is understood to be morally right depends (at least in part) on one's **identity** as a moral person
- **Premise 2:** Ones' *race, ethnicity* and *culture* is central to one's identity as a moral person
- **Conclusion:** Thus, what is understood to be morally right by an individual depends (at least in part) on that person's race, ethnicity, and culture

Cultural Diversity and Ethics

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- Ethics increased recognition, interest and respect for diverse societal values. **Why?**
- Fundamental question: What place, if any, do race, ethnicity, and culture have in moral theory?



2. Ethical Principles and Moral Values

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Ethical Principles

- **Principle:**
 - It is a ‘standard that is to be observed’
- **Ethical Principles;**
 - Basic ideas that are starting points for understanding and working through a problem.
 - Tents that are important to uphold in all situations
 - Presuppose that health professionals should respect the value and uniqueness of persons and consider others to be worthy of high regard.

Why Ethical Principles?

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- Conflict is inevitable
 - Ethical principles provide the framework/tools to resolve conflict in a fair, just and moral manner

Principles of Healthcare (Biomedical) Ethics

- Autonomy
- Beneficence
- Non-Maleficence
- Justice
- Informed consent (IC)
- Confidentiality
- Veracity
- Fidelity

I. Autonomy (Respect for persons)



- Comes from Greek word “**auto**” and “**nomos**”
 - ‘**Auto**’: Self” and ‘**nomos**’: Rule” or “law”- **Self Rule**
- The freedom to make decisions about oneself
- The right to participate in and decide on a course of action without undue influence
- The right of individuals to **self rule/self-determination**
- It is respecting the decision-making capacities of autonomous persons;
 - Enabling individuals to make reasoned informed choice

Autonomy....

- Autonomy requires decision making capacity
 - Lack should be proven not assumed
- HCPs need to respect patient's rights to make choices about healthcare, even if the healthcare providers do not agree with the patient's decision
- Two important conditions must be met for the autonomy
 - ✓ Are pts. competent to make decisions for themselves?
 - ✓ Are pts. free from coercion in making the decision?

Autonomy....

- **Example: Informed consent (IC)**
 - **IC:**
 - It is a **process** by which patients are **informed** of the possible outcomes, alternatives and risks of treatments and are required to give their **consent freely**
 - It assures the legal protection of a patient's right to personal autonomy in regard to specific treatments and procedures

Autonomy....

- **Consent**, in medical cases, means that a patient:
 - Has knowledge of the **nature** or **extent of the harm or risk**
 - **Appreciates** and **understands** the nature of the harm or risk
 - Has **consented** to the harm or assumed the risk and
 - **Understands** that the consent is **comprehensive**, i.e. extends to the entire procedure, including its consequences
- Consent in the case of minors

Autonomy....

- **Minors:**
 - Usually regarded as a person under the age of 18 years who requires the consent of his or her parent or guardian to enter into certain legal agreements.
- **Discussion?**
 - 1. List circumstances where consent of parent or guardian is not required?
 - 2. How informed consent is obtained in the case of mentally ill persons?
 - 3. When is emergency treatment justified?

Autonomy....

- Answer:
 1. Minors over 18 years of age are competent to consent:
 - If the parents or guardian cannot be found, or refuse permission for treatment
 - If the medical treatment is necessary to preserve the life of the child or to save the child, and that the need is so urgent
 2. From patient's spouse, parent, major child or brother or sister

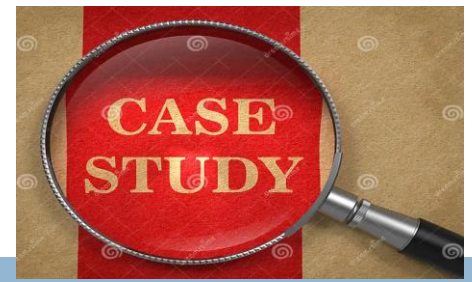
Autonomy....



3. Emergency treatment justified;

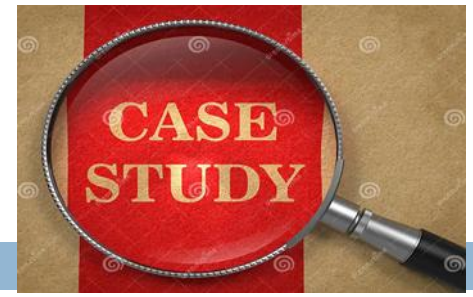
- If there is an emergency
- The patient is or has been unable to communicate
- The treatment is not against the patient's will
- The treatment is in the best interests of the patient
- Where an operation is extended to save the patient's life

Autonomy: Case scenarios



- **Case 1:** A doctor at a certain public hospital is treating an eight year old boy who requires a blood transfusion. For religious reasons his parents are not prepared to give consent.
- **What should the doctor do?**

Autonomy: Case scenarios



- **Case 2:**
 - A doctor has diagnosed her patient as suffering from cancer. She knows that her patient is subject to bouts of depression and that if she informs the patient that she is suffering from cancer, she will go into a deep depression that will undermine the treatment.
 - **Should the doctor reveal the diagnosis in order to obtain an informed consent to treat the condition?**

2. Non-Maleficence (Doing no harm)



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- “*First, do no harm*” principle
- Avoid causing deliberate **harm, risk of harm**, and harm that occurs during the performance of beneficial acts.
- Actions are wrong as they induce bad consequence
- Prohibits both deliberate and unintentional harm
- All treatment involves some harm, even if minimal;

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- But the harm should not be disproportionate to the benefits of

Non-maleficence..

- Be aware of the doctrine of double effect:
 - Where a treatment intended for good unintentionally causes harm.
 - **Example 1:** A pharmacist who fails to counsel patients, for fear that the patients will discontinue using a drug if the info is disclosed, is adhering to principle of non-maleficence

Non-maleficence..

- **Example 2:**
 - Killing and letting die (active and passive euthanasia)
 - Withholding or withdrawing life-sustaining treatment. (O2, Glucose , IV drug)
 - Providing high dose of steroids to an asthmatic patient
- **Example3:** Experimental research that have negative consequences on the client

3. Beneficence (Doing good)

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- Refers to actions that promote the wellbeing of others
- It is the basis for all health care providers
- Stress the importance of acting in the patient's best interest
- It stipulates that ethical theories should strive to achieve the greatest amount of good because people benefit from the most good
- Principle of beneficence has three components:
 - ❖ Promote health
 - ❖ Prevent harm
 - ❖ Remove evil or harm

Beneficence...

- Actions are right if they produce benefits
- **Example:**
 - The pharmacist who neglects to counsel patients is violating the principle of beneficence; b/c info about medications generally helps patients use their medications the correct way

Beneficence: Case Scenario:

- A doctor works in a mission hospital in a rural part of the country where the incidence of HIV-positive people is very high. She leaves to drive to the city on the weekend to attend her son's wedding. As she is off duty and expecting to relax she leaves behind her medical bag which contains universal precautions against HIV infection.

Beneficence...

- While she is still in the high risk area, an hour's drive away from the mission hospital, she comes across a bus accident with bodies lying scattered in the road. She is feeling overworked, and also needs to hurry to reach the city in time for her son's wedding. She decides not to stop but uses her cell phone to call the mission hospital with a request that they send help immediately.
- Three people die from loss of blood which could have been prevented if she had stopped and assisted

66 1. Did the doctor act ethically? Why or why not?

2. Did the doctor act legally? Why or why not?

4. Justice (Fairness)



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- It involves the fair distribution of care
 - It is fair, equitable and appropriate treatment
- It is the basis for the obligation to treat all clients in an **equal** and **fair** way.
- Address questions of distribution of **scarce resources**
- Decision is based on client need and fair distribution of resources
- It is complex ethical principle:
 - Range from the fair treatment of individuals to the equitable allocation of healthcare dollars and resources

Justice...

- Equitable distribution of burdens and benefits
- Be able to justify your actions in every medical situation
- **Two major categories:**
 - **Procedural justice & distributive justice**
 - **Procedural justice** : Asks where **fair procedure in place** and where those procedures followed?
 - Due process
 - **Example:** keeping your turn in order to get your primary physician

Justice...

- **Distributive justice:**
 - Concerned with the **allocation of resources**
 - Form of *rationing* of the delivery of health care services.
 - All involved should have equal entitlements, equal access to benefits and burdens
 - Dividing burdens and benefits
 - Each person should get an equal share of the burdens and benefits unless there is a material reason to discriminate
 - **Example:** How to determine who qualifies for one available kidney?
 - 55 yrs old man with three children Vs a 13 yrs old girl?

Justice...

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■ **Comparative Justice**

- Making a decision based on criteria and outcomes

■ **Non-comparative Justice**

- Method of distribution using lottery system
- Any health professional should treat all the patients equally irrespective of sex, culture, race, religion, language, social status and political status

Justice...



- Society uses a variety of factors as a criteria for distributive justice, including the following:
 - To each person an equal share
 - To each person according to need
 - To each person according to effort
 - To each person according to contribution
 - To each person according to merit
 - To each person according to free-market exchanges

5. Veracity (Truth telling)

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- Obligation to full and honest disclosure
 - ✓ Health care providers obliged to be honest with clients
- It is essential to the integrity of the client-provider relationship
- The patient has the right to the truth to;
 - ✓ Medical condition
 - ✓ Course of the disease
 - ✓ Treatments

Veracity...

- **Code of ethics** states:
 - A pharmacist: "has the duty to tell the truth and to act with conviction of conscience"
 - **Example:**
 - If you have been informed about the result of an HIV test taken by someone in your community who then asks to know his/her result, you should tell the person the truth even though this might be very upsetting to that person.

6. Informed Consent (IC)

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- **Consent:**
 - Agreement, compliance or permission given voluntarily without compulsion
- **IC:**
 - It is an agreement to a certain course of action, such as treatment or participation in research, on the basis of complete and relevant information by a competent individual without coercion

Informed Consent...

- Requires honesty and autonomy to exist
- A right of client to be informed and to freely choose a course of action
- It includes the following:
 - Nature of ailment or disease process
 - Nature and plan of proposed line of treatment
 - Alternative form or line of treatment
 - Nature of risks involved in both
 - Relative chances of success, benefit, burden or failure
 - Freedom of choice

Informed Consent...

- Informed consent exists when:
 - All **relevant information** has been provided
 - The patient **understands** the information
 - Patient is **capable** of understanding the info
 - Consent is **freely** given and there is **no coercion**

7. Confidentiality



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★ Comes from Latin fide: **trust**

★ Show trust by imparting secrets; tell in assurance of secrecy

★ Based on loyalty and trust

★ Confidentiality in the health care context:

★ The requirement of HPs to keep information obtained in the course of their work

Confidentiality...

- ★ From patients perspective:
 - “Self-disclosure” and they should be the ones making this decision
- ★ Clients right to give/refuse consent to release information

Confidentiality...

- The main ethical issue surrounding confidentiality is whether more harm is done by occasionally violating its mandate or by always upholding it, regardless of the consequences.
 - This limitation to confidentiality is known as the *Harm Principle*
- Unfortunately, breaches of confidentiality more often result from careless slips of the tongue than from rational decision making.
 - Such social trading in gossip about patients is unprofessional, unethical, and, in certain cases, illegal.

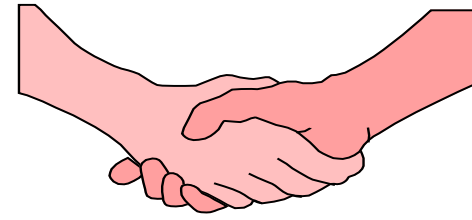
Confidentiality...

- Maintain the confidentiality of all personal, medical and treatment information
- Information to be revealed with consent and for the benefit of the patient
 - Except when ethically and legally required
- Disclosure should not be beyond what is required

Confidentiality...

- **When confidentiality may be breached?**
 - There is a professional duty to maintain confidentiality unless:
 - A court of law orders them to make a disclosure, (e.g. in a paternity dispute)
 - An Act of Parliament requires them to make a disclosure, (e.g. reporting child abuse)
 - There is a moral or legal obligation on the doctor to make a disclosure to a person or agency that has a reciprocal moral or legal obligation to receive the information, (e.g. where a patient threatens to kill someone)
 - The patient consents to the disclosure being made

8. Fidelity



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- Strict observance of promises or duties
- Being faithful to one's commitments and promises
- The right of patients to have practitioners provide services that are in the patients best interest
- **Example;**
 - Recommending vitamins patients don't need
 - Failing to confront prescriber with inappropriate Rx out of fear that s/he will direct patients elsewhere

Fidelity: Case scenario

- A pharmacist is consulted by a patient who has been diagnosed as HIV positive. The patient is married but is having sexual intercourse with another woman with whom he is having an affair. The doctor advises the patient that the latter should tell both his wife and the other woman that he is HIV-positive and to ensure that precautions are taken.

Fidelity: Case scenario...

- For religious reasons the patient is not prepared to use a condom. He also does not wish to inform his wife because she may divorce him, and does not want the other woman to know in case she ends their relationship.
- **Question?**
 - What should the pharmacist do?



Framework For Ethical Analysis

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Code of Ethics

Code of Ethics for Pharmacists

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- Principles that form the fundamental basis of the roles and responsibilities of pharmacists
- They form a foundation pharmacists' professional behavior, attitudes , and actions
- Guide pharmacists in relationship with;
 - ✓ Their job
 - ✓ Patients/clients
 - ✓ Health professionals
 - ✓ Society/public ...

Why Code of Ethics???

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- To define accepted/acceptable behavior
- To promote high standards of practice
- To provide a benchmark for members to use for self evaluation
- A framework for behavior and responsibilities
- To foster good relationship (Phar, Pt, other HPs)
- As a vehicle for occupational identity
- As a mark of professional maturity



Code of Ethics for Pharmacists Practicing in Ethiopia

Pharmacist in relation to his/her patient

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- Always follow patient first or service first rule
- Therefore:
 - Provide the service only when needed
 - Provide the required service to the best of one's knowledge
 - Be compassionate, confident and impartial
 - Should not abuse his/her relationships for personal gains

The Pharmacist in relation to the public

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- Always provide quality service
- Therefore:
 - ▣ Maintain good personal quality:
 - ✓ Be on duty with clear mind
 - ✓ Be presentable
 - ✓ Master good communication skills
 - ✓ Be knowledgeable
 - ✓ Provide accurate and appropriate info
 - ✓ Always update yourself

Public ...

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- ▣ Maintain standard premises
 - ✓ Clean environment
 - ✓ Paint/color/lighting
 - ✓ Controlled substance in a safe
 - ✓ Price clearly labeled
- ▣ Strive to create public awareness on:
 - ✓ Rational drug use
 - ✓ The profession in general

Public ...

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- ▣ Be accountable and liable:
 - ✓ For one action and;
 - ✓ For services rendered under his/her supervision
- ▣ Keep the secrecy of all confidential documents
- ▣ Not be involved in clinical trials that do not get prior ethical clearance

Public ...

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- ▣ Immediately report any AEs when the volunteer's health is in danger (involved in clinical trials)
- ▣ Not participate in the use of drugs for the purpose of *euthanasia in human being*

Pharmacists as member of the community

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- Be good citizen !
- Be a person of high principles and integrity
- Be law abiding citizen
- Conform to positive norms and values of the society
- Actively participate in community services and social development activities
- Address the needs of special groups society such as under aged, disabled and mentally ill people
- Always maintain high standards of professional conduct

Pharmacists in relation to fellow professionals

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- Always: respect, support, cooperate and correct
- Have the responsibility to expose an act of misconduct or malpractice
- Actively participate in professional association and scientific societies with objective of promoting profession

Pharmacists in relation to other professionals

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- **Things to do:** respect, support, cooperate, communicate and correct
 - ✓ Respect the reputation of the prescriber to his/her patient
 - ✓ Expose deliberate act of malpractice through appropriate channel
 - ✓ Give professional consultancy on the appropriate use of pharmaceuticals
 - ✓ Strive for teamwork spirit and promotion & expansion of health service

Relation to other professionals....

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- **Things not to do:** Favourism, partiality, and strings
 - X Recommend a particular medical practitioner to patient
 - X Enter to any secret arrangement or negotiation with a medical practitioner
 - X Collaborate with health professional for malpractice

Pharmacists in relation to his/her job

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- Primary concern should be to promote the health and well being of the patient and the public
 - ▣ Practice his/her profession after being registered and/or licensed
 - ▣ Give comprehensive service in his/her profession.
 - ▣ Not delegate his/her subordinate tasks, which should be performed personally.
 - ▣ Provide current drug info to the public & health professionals.

Pharmacists in relation to his/her job...

- ▣ Immediately report any observed unwanted reactions, side effects, and adverse reactions.
- ▣ Carry his/her responsibility with in his/her professional competence.
- ▣ Update his/her knowledge & maintain professional competence

Pharmacists in relation to management and utilization of pharmaceuticals

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- Manufacture pharmaceuticals of good quality, safety, and efficacy
- Don't make available for sale substandard, counterfeit, deteriorated and mislabeled pharmaceuticals
- Confirm legibility of prescription
- Accept prescription only in written form except in emergency
- Give written and oral advise and counseling

- ❑ Don't fill prescription of drugs written for prescriber for his/her personal use
- ❑ Not dispense prescription drugs without prescription
- ❑ Properly procure, store, distribute and use drugs
- ❑ Keep records properly
- ❑ No self advertise contrary to the regulation
- ❑ Don't promote products of a particular manufacturer

Pharmacists in relation to his/her trade

59

- Don't promote unethically
- Promote fair distribution of pharmaceuticals
- Don't put for sale medical samples
- Obtain pharmaceuticals from genuine and legal source
- Have strong moral obligation not to come under commercial pressure



Pharmacist and Patient Relationship

Paternalism Vs Consumerism

History of the Patient-Pharmacist Relationship

Previous trend

- Lower rates of education
- Less access to medical care
- Provider trusted completely
- Provider advice/medication and patient would take it
- Patients' ideas, concerns & expectations not asked about
- **Provider-Centred Approach**

Current trend

- Higher rates of education
- Medical advice on internet/libraries
- Better access and choice of good medical care
- High profile cases of providers not acting professionally- general mistrust of providers
- Pts want to make own decisions

Pharmacist -Patients relationships

- Pharmacist-patient interaction is shaped by their differing **social roles** and their different **needs**

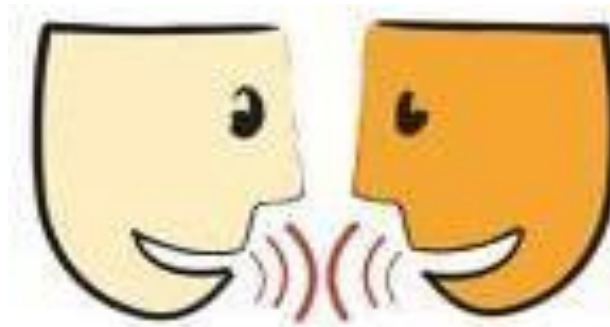
Pharmacist - Patient interaction

Social roles

needs

What is the pharmacist-patient relationship?

- Certain **sensitive** relationships involving unequal parties demand a **higher motivation** than **self-interest**
- Central in P-P encounter is **communication**:
 - **Exchange of information**



Types of P- P Relationship

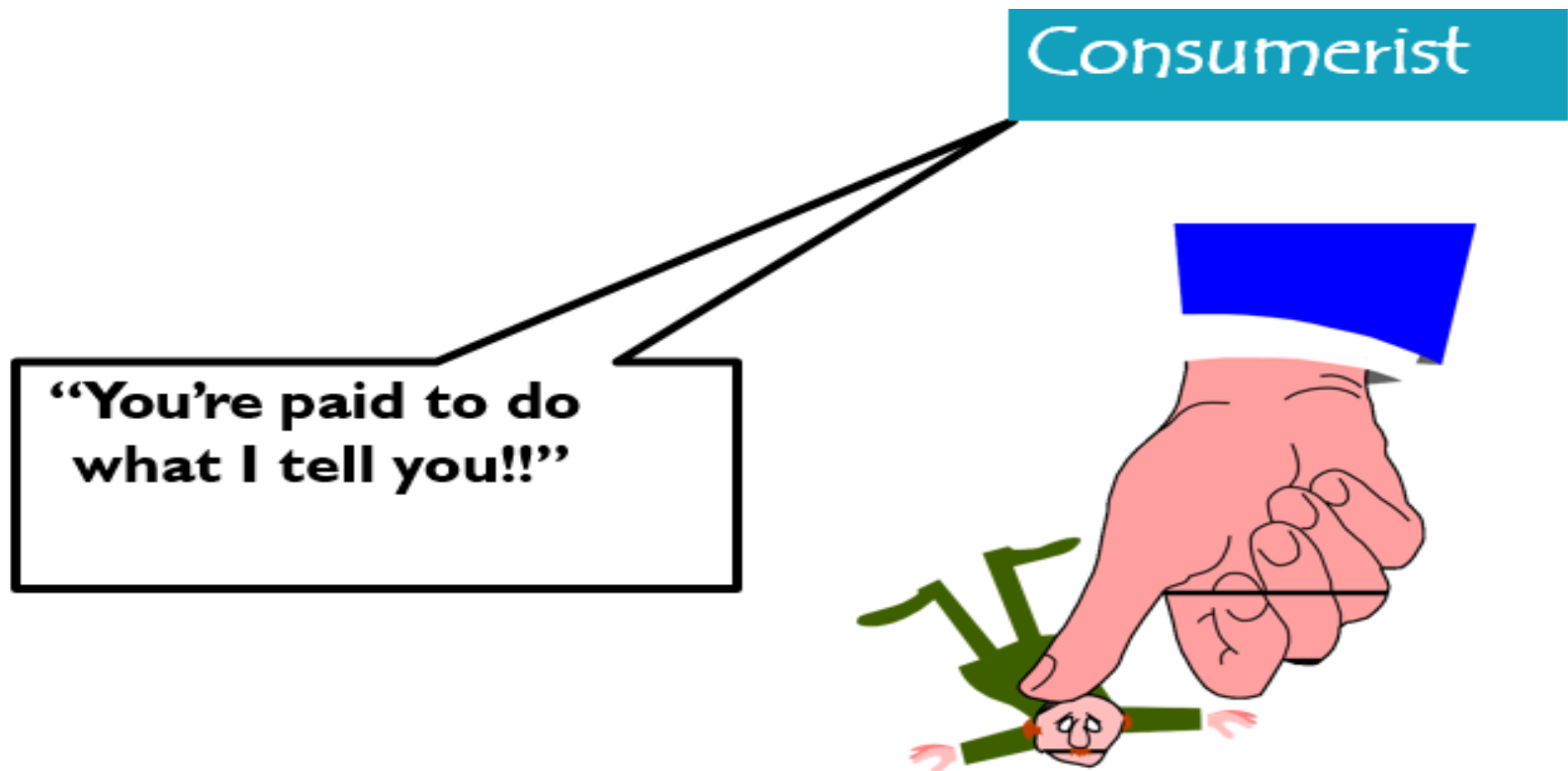
<u>Patient Control</u>		<u>Pharmacist control</u>	
Low	High	Low	High
		Default	Paternalist
		Consumerist	Mutuality



Consumerist Approach

- Less authoritarian
- Patients take the active role and pharmacist passive
- Patient controlled consultation
- “Buyer and seller” relationship

Consumerist....



Paternalistic approach

- ➔ Autocratic model
- ➔ Patient submissive
- ➔ Traditional P-P relationship
- ➔ Dominant pharmacist



What can be done?



- ★ **Cultivate patient-centered partnership**

- ♥ Mutuality

- ★ “The patient desires to be known as a human being, not merely to be recognized as the outer wrappings for a disease”.

Assignment

Compare and contrast the Ethiopian code of ethics for pharmacists with FIP code of ethics

Ethical Issues in Healthcare

Ethical issues in healthcare

- Ethical situations in health care can be divided into two broad categories:
 - “*Macro*” or “*Micro*” Ethical situations
- **Macro situations:**
 - Involve issues that are **not specific to a given health care practitioner**
 - Issues that must be **addressed by all health care practitioners** and society in general

Ethical issues in healthcare...

- Macro ethical situations includes issues such as;
 - Abortion
 - Assisted suicide
 - Genomics
 - Rationing and access to health care
 - Organ transplantation, and
 - In vitro fertilization

Ethical issues in healthcare...

- **Micro situations:**
 - Issues that **may confront individual practitioners** in the course of their daily practice.
 - In the case of pharmacists , these may include the use of placebos, patient confidentiality and informed consent.

Case scenario

- What will be your decision if the medication is a **controlled substance** used for pain control in a critically ill patient but if there is no prescription order for the drug?
 - *I will provide the medication even if it is illegal vs I do nothing*

Ethics of Health Disparities



- Healthcare's major influences
 - Disability, death
 - Pain and Suffering
 - Quality of life

Health Disparities...

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- Disparities in health status and healthcare
 - Tragic, horrendous
 - Moral problems of the first order
 - Ethically unacceptable

Health Disparities: Source of ethical dilemma:

64

→ Demand Vs. Supply of Pharmaceuticals

- Improving access to care
- Allocating limited medications and donor organs
- What is needed? Rationing?



Equity

Utility/Efficiency

Utility Considerations : Maximize Benefit

65

- Likelihood of benefit for a vaccinated person (for ex. age)
- Likelihood of benefit for community in terms of disease burden
- Maintain health care services (priority to HWs)
- Maintaining essential services and minimize societal and economic impact (priority to essential functions)

Equity Considerations

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- **Might sometimes conflict with utility**
- Priority to the worst-off (for ex. economically)
 - ✓ Vaccine - poorest countries first
- Vulnerable populations may need more resources
- Priority to those at greatest risk of dying

Examples: Priority for health care workers

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- **Utilitarian approach:** HWs should have priority
- Reciprocal obligation of policy-makers to give priority
- Difficulties in exact definition of HWs:
 - Health care professions vs. other professions
 - Volunteers, NGOs etc...

Examples: Age as a Priority Criterion

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- Equity Argument: Priority to children (vulnerable)
- Utility Argument (QALYs, DALYs)
 - ✓ Life-cycle principle – save the most life years
 - ✓ Potential priority to infants
- Culture dépendent: Require public consultation

In any case Priority setting should:

- Respect human rights
- Not discriminate against individuals
- Public Communication on priority criteria

Model: Ethical/Moral Practice

70



Respect - Beneficence - Justice

Crucial: Intersectional Approaches

71

Stakeholders
Communities
Disciplines
Professionals
Institutions & Organizations

Respect - Beneficence - Justice

Common ethical issues in healthcare

- **Beginning of life Issues:**
 - **Contraception:** Requests for contraceptives from minors and explaining the risks of different methods of contraception.
- **Assisted reproduction:** For couples (individuals) who cannot conceive naturally, there are various techniques of assisted reproduction, such as;
 - **Artificial insemination(AI):** is the manual placement of semen in the reproductive tract of the female by a method other than natural mating
 - **In-vitro fertilization:** From July 25 1978 with the birth of Louise Joy Brown – up to Now >4 million babies are born with IVF

Common ethical issues in healthcare...

- **Natural fertilization Vs In vitro fertilization**

- **Embryo transfer:** process by which an embryo is collected (flushed) from one female (donor) and transferred to another female (recipient) to complete the gestation period.
- **Surrogate or substitute gestation:** A gestational carrier (surrogate mother) is a woman who agrees to have an embryo generated from the sperm of a man through the process of IVF who is not her husband.
 - A gestational carrier's pregnancy does not involve her own egg. (religion, culture, norm....)

Common ethical issues in healthcare ...

- **Prenatal genetic screening:**
 - Genetic tests are available to determine the embryo or fetus genetic abnormalities and whether it is male or female
 - Depending on the findings, a decision can be made whether or not to proceed with pregnancy
- **Abortion:**
 - This has long been one of the most divisive issues in medical ethics, for health professionals, religious leaders and for public authorities.

Common ethical issues in healthcare ...

- **Questions?**
 - Do you agree on the legalization of abortion in Ethiopia?
 - When is abortion justified?
 - Rape, pregnancy from relatives, risk for the mother health

End-of-Life Issues

72

Death with Dignity or Murder?



Question??

73

Do human beings have the right to make decisions on when to end their lives?

The Hippocratic Oath: "I will neither give a deadly drug to anybody who asked for it, nor will I make a suggestion to this effect."

- **Euthanasia:**
 - Derived originally from to Greek words meaning simply **“good death”**
 - Knowingly and intentionally performing an act that is clearly intended to end another person’s life
 - Death caused by an intentionally lethal dose of medication ordered by a legal prescriber and administered by a health care professional(s)
 - Direct and intentional medical killing
 - Can be voluntarily or involuntarily

Assisted Suicide and Euthansia...

- **Voluntary euthanasia:**

- Occurs when the patient requests someone to give him/her a lethal drug to bring about death.

- **Involuntary euthanasia:**

- Occurs when the patient has not given consent or requested anyone to give him/her a lethal drug.
- Rather, the person initiating the euthanasia believes it to be the wish of the patient to be euthanized, and takes matters into his/her own hands.

- **Consent from guardian**

Types of Euthansia

- **Direct euthanasia:**
 - Occurs when the occurrence of death is directly established.
- **Indirect euthanasia:**
 - Occurs when death is hastened in some way, either by withholding treatment or by giving an overdose of medicine.

Types of Euthansia ...

- **Active euthanasia:**

- To act in a positive way (comission) in order to end a person's life with the aim to spare them, the patient, pain and suffering as well as an undignified death
 - Introducing something to cause death

- **Passive euthanasia:**

- The act of omission (not acting) where the person does not do anything to save the patient's life
 - Withholding treatment or supportive measures

Assisted suicide...

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□ Assisted suicide:

- General term for helping a patient to terminate their life
- Although medical euthanasia (“mercy killing”) has long been an ethical issue, it has only been in recent years that the question of **assisted suicide** has been examined
- From an ethical perspective, the key issue remains whether assisted suicide violates the Hippocratic responsibilities of health care practitioners to “**do no harm**”

Assisted suicide...

- Those who advocate its availability to patients suggest that allowing a patient to continue to experience persistent pain is “**to do harm**”



Types of Assisted Suicide: two procedural classifications of euthanasia

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Euthanasia

- Physician prescribe the treatment
- Professional directly administers the treatment to the patient

Professional-Assisted Suicide

- Physician prescribe the treatment
- Patient but NOT providers administers the treatment

Types ...

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Active

- Administering treatment to end life

Passive

- ➔ Withholding from treatment that sustains life

Voluntary

- ➔ Patient consents to treatment

Involuntary

- Patient is unable to consent to treatment

Legality of Euthanasia and assisted suicide

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■ Euthanasia:

- Illegal in **Ethiopia**, USA, Canada
- Legal in Belgium, Netherlands

■ Assisted suicide:

- ✓ Illegal in **Ethiopia**
- ✓ Legal in some states of USA (Oregon, Washington...), Belgium, Netherlands



Example: Professional-Assisted Suicide

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Oregon's Death With Dignity Act

1. Requires the patient give a fully informed, voluntary decision.
2. Applies only to the last 6 months of the patient's life.
3. Makes it mandatory that a second opinion by a qualified physician be given that the patient has fewer than 6 months to live.
4. Requires two oral requests by the patient.
5. Requires a written request by the patient.
6. Allows cancellation of the request at any time.
7. Makes it mandatory that a 15-day waiting period occurs after the first oral request.
8. Makes it mandatory that 48-hours (2 days) elapse after the patient makes a written request to receive the medication.
9. Punishes anyone who uses coercion on a patient to use the Act.
10. Provides for psychological counseling if either of the patient's physicians thinks the patient needs counseling.
11. Recommends the patient inform his/her next of kin.
12. Excludes nonresidents of Oregon from taking part.
13. Mandates participating physicians are licensed in Oregon.
14. Mandates Health Division Review.
15. Does not authorize mercy killing or active euthanasia.

Suicide Tourists Make Swiss Uneasy as Terminally Ill Seek Death

80

By Klaus Wille - Jan 19, 2011 2:01 AM GMT+0300

[Switzerland](#) is the destination of choice for people from abroad who want to die. The office of the country's top legal official is pushing to change that. While assisted suicide is permitted in the Netherlands, [Belgium](#) and the U.S. states of Oregon, Washington and [Montana](#), only Switzerland allows doctors to help foreigners end their lives. More than 25 percent of the 380 assisted suicides in Switzerland during 2009 involved foreigners, most of whom died after drinking water laced with a lethal dose of barbiturates.



<http://www.bloomberg.com/news/2011-01-18/suicide-tourists-make-swiss-minister-uneasy-as-terminally-ill-seek-escape.html>

Former Justice Minister Eveline Widmer-Schlumpf has proposed making the practice more difficult by demanding oversight by doctors who aren't connected with one of the country's four right-to-die organizations. Photographer: Fabrice Coffrini/AFP/Getty Images

Why Assisted Suicide?

81

- Experience of a tragic death
- Witness of terrible suffering
- Fear about suffering and pain
- Fear of being a burden
- Depression at end of life
- Fear about loss of control and dignity

Controversies on euthanasia & Assisted suicide

- Various arguments are commonly cited for and against euthanasia and physician-assisted suicide.
- **Arguments for**
 - **Freedom of choice**: Advocates argue that the patient should be able to make their own choice.
 - **Quality of life**: Only the patient really knows how they feel, and how the physical and emotional pain of illness and prolonged death impacts their quality of life.
 - **Dignity**: Every individual should be able to die with dignity.

Controversy...

- **Witnesses:** Many who witness the slow death of others believe that assisted death should be allowed.
- **Resources:** It makes more sense to channel the resources of highly-skilled staff, equipment, hospital beds, and medications towards life-saving treatments for those who wish to live, rather than those who do not.
- **Humane:** It is more humane to allow a person with intractable suffering to be allowed to choose to end that suffering.
- **Loved ones:** It can help to shorten the grief and suffering of loved ones.

Controversy...

- **Arguments against**

- **The doctor's role:** HCPs may be unwilling to compromise their professional roles, especially in the light of the Hippocratic Oath.
- **Moral and religious arguments:** Several faiths see euthanasia as a form of **murder** and morally unacceptable.
 - Suicide, too, is "illegal" in some religions. Morally, there is an argument that euthanasia will weaken society's respect for the sanctity of life.
- **Patient competence:** Euthanasia is only voluntary if the patient is mentally competent, with a lucid understanding of available options and consequences and the ability to express that understanding and their wish to terminate their own life.

Controversy...

- **Guilt:** Patients may feel they are a burden on resources and are psychologically pressured into consenting.
 - They may feel that the financial, emotional, and mental burden on their family is too great.
 - Even if the costs of treatment are provided by the state, there is a risk that hospital personnel may have an economic incentive to encourage euthanasia consent.
- **Mental illness:** A person with depression is more likely trusted Source to ask for assisted suicide, and this can complicate the decision.

Controversy....

- **Slippery slope:** There is a risk that physician-assisted suicide will start with those who are terminally ill and wish to die because of intractable suffering, but then begin to include other individuals.
- **Possible recovery:** Very occasionally, a patient recovers, against all the odds.
 - The diagnosis might be wrong.
- **Palliative care:** Good palliative care makes euthanasia unnecessary.
- **Regulation:** Euthanasia cannot be properly regulated.

Ethics Issues in Pharmaceutical R &D

82

■ Source of Ethical dilemma:

- Problem solving research Vs. Research for personal gain
- Patient and public may have different understanding of the nature, purpose and priorities of research than held by the researcher
- Participants of research are liable to harm in research processes
- Assuring the liability of the research

Historical Perspectives

83

Nazi War Crimes and the Nuremberg Code

- Prior to and during the World War II (1940s): experiments done on prisoners **without their consent**
- Many involved exposing subjects to extreme suffering for observational purposes such as:
 - Determine **time to death**: submerged in **freezing water**

Historical: Nazi War Crimes

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➡ Determine ability of subjects to withstand high altitude

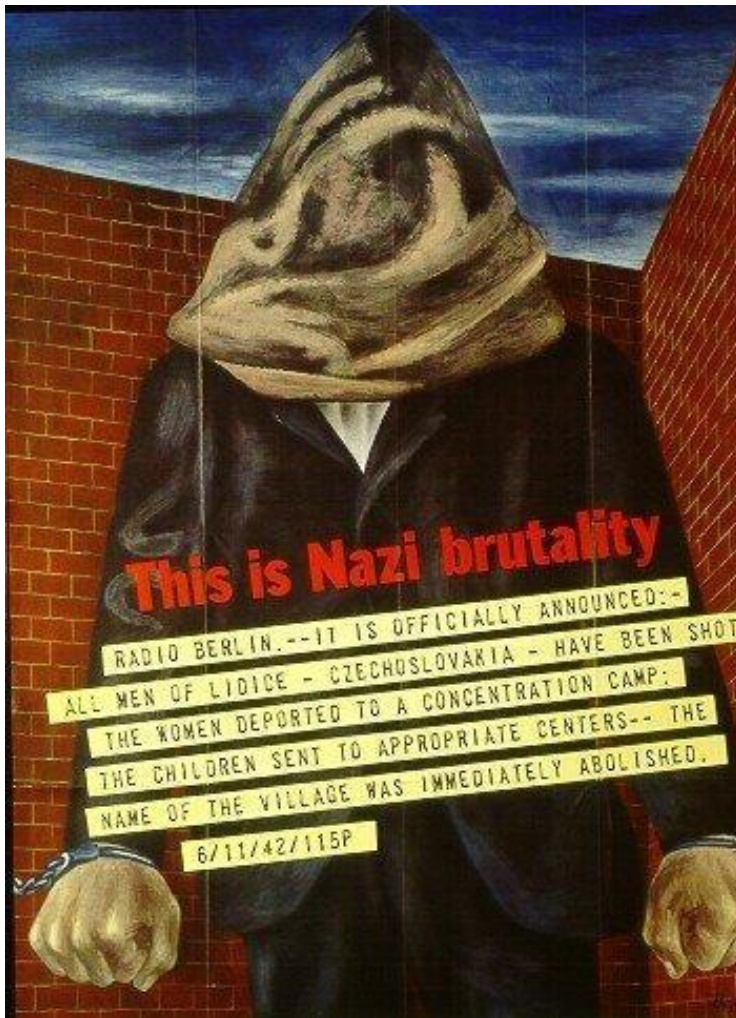
➡ Conditions: forcing them **into low-pressure chambers**

- Other experiments

- ➡
 - Deliberately inflicting wounds or burns
 - Infecting subjects
 - Poisoning them
 - To test various treatments or to perform autopsies afterwards....

Echo from Past: Nazi War Crimes

85



Historical...Nazi War Crimes

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- Nuremberg Military Tribunals at the end of WW-II:
 - Scientists were held accountable for their actions
 - Many issues in research ethics received public scrutiny
 - Among the charges were that German physicians conducted medical experiments on thousands of concentration camp prisoners **without their consent**

Historical...Nazi War Crimes

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The Nuremberg Code (1947):

- ♥ Was developed in response to the medical experiments conducted under the Nazi regime
- ♥ Main focus was on protecting research participants from harm and ensuring that they had given valid consent.
- ♥ 10 principles
- ♥ Famous principle: "**Voluntary consent** of the human subject is absolutely essential"

Historical...

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The Tuskegee Syphilis Study (1942-1972)

- Well known chapter in history occurred during a research project conducted by the U.S. Public Health Service
- In 1932 the U.S Public Health Service (PHS) began a study of **the natural course of untreated syphilis among rural black males** in Macon Country, Alabama 400 men with syphilis and 20 uninfected controls enrolled
 - “Tuskegee Study of Untreated Syphilis in the Negro Male.”
- **No Informed consent!!**

Historical...

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- When penicillin became widely available by 1947 as a standard cure for syphilis, treatment was denied to these subjects
- Study was halted in 1972 when national press reports prompted public outrage
 - The experiment was stopped in 1972 as “**ethically unjustified**” when the media exposed it to public scrutiny.
- This was the longest non-therapeutic experiment on human beings in medical history

Historical...

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- By the time the study was stopped:
 - Dozens of the participants had died of syphilis
 - Many of their wives had been infected
 - Many children born with congenital syphilis
- President Clinton formally apologized for the study's injustices on behalf of the U.S government in 1997

Historical...

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- **Belmont Report**

- Promulgated by US National Commission for the Protection of Human Subjects of Biomedical and Behavioral Research in 1979
- Addressed the three basic ethical principles, i.e., **respect for persons, beneficence & justice**
- Identified three components of informed consent: **information, comprehension and voluntariness**

National Health Research Ethics Review

Guidelines of Ethiopia

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- Formulated in 1995 and revised in 1997 and 2004
- Amended on several occasions in order to accommodate the new ethical challenges
- They are intended to:
 - Develop the capacity of health researchers in health research ethics
 - Serve as a guide for the various ethics review committees of different levels in the country

Basic guidelines for Research

93

- **Respect for person**

- Autonomy of the person to participate or not

- **Beneficence**

- Constraints and presumed risks must be accepted for patients included in the study

- **Justice**

- Vulnerable subjects should not participate in the study unless under strict conditions to protect them

- **Drug formulary:**

- It is a list of drugs that are approved for use either within an institution or for reimbursement by a third-party payer.
- The restrictive nature of formularies has led to a number of important ethical questions.
 - Does the use of generics violate the autonomy of the patient or prescriber?
 - Is the use of such substitution a violation of informed consent?
 - Does the use of formularies violate the ethical principles of beneficence and non-maleficence?

- 
- However, one could argue that the use of formularies and the resultant constraint on drug costs helps to ensure affordable access to quality health care for more people (distributive justice).